



**NATIONAL CENTER
FOR COMMUNICABLE DISEASES**



Country perspectives:

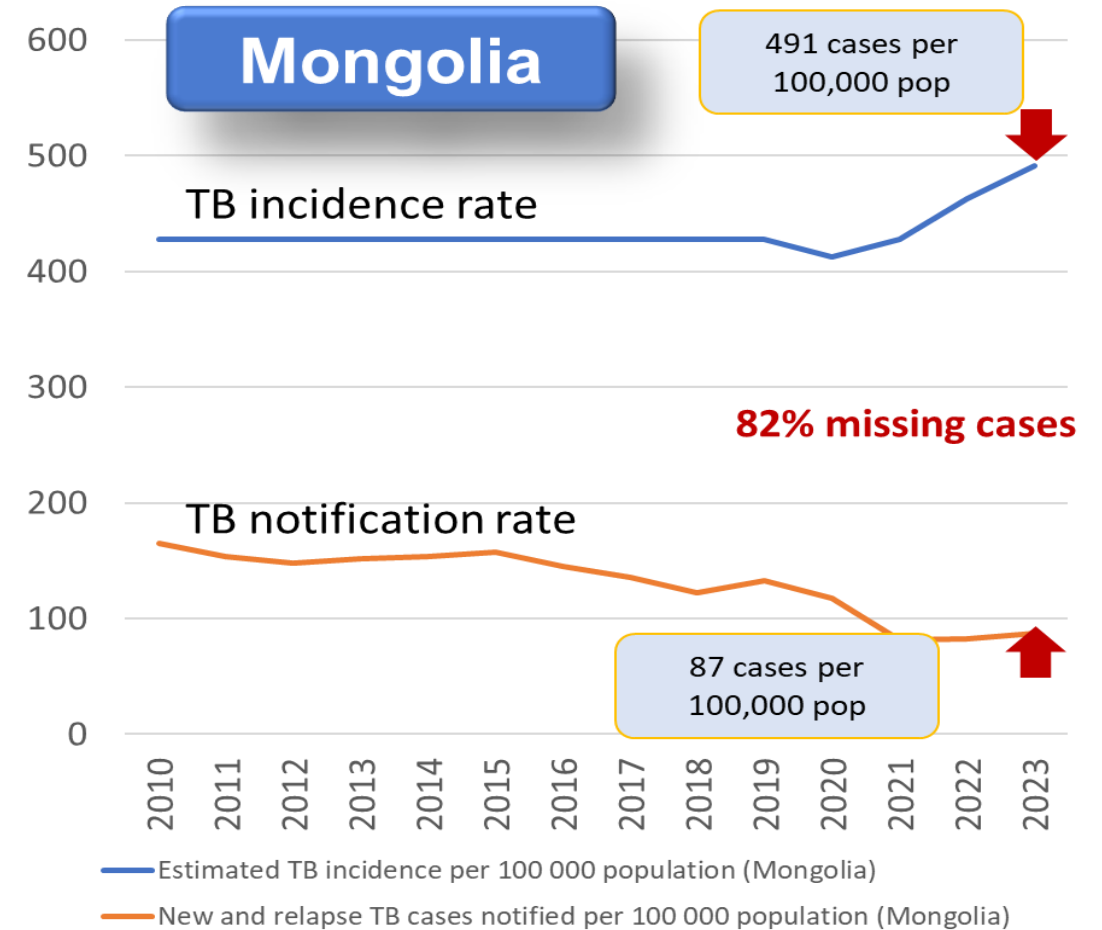
Harnessing synergies and opportunities between TB and PHC in Mongolia

G.Gantungalag,
NTP manager

TB situation in Mongolia

2023 data:

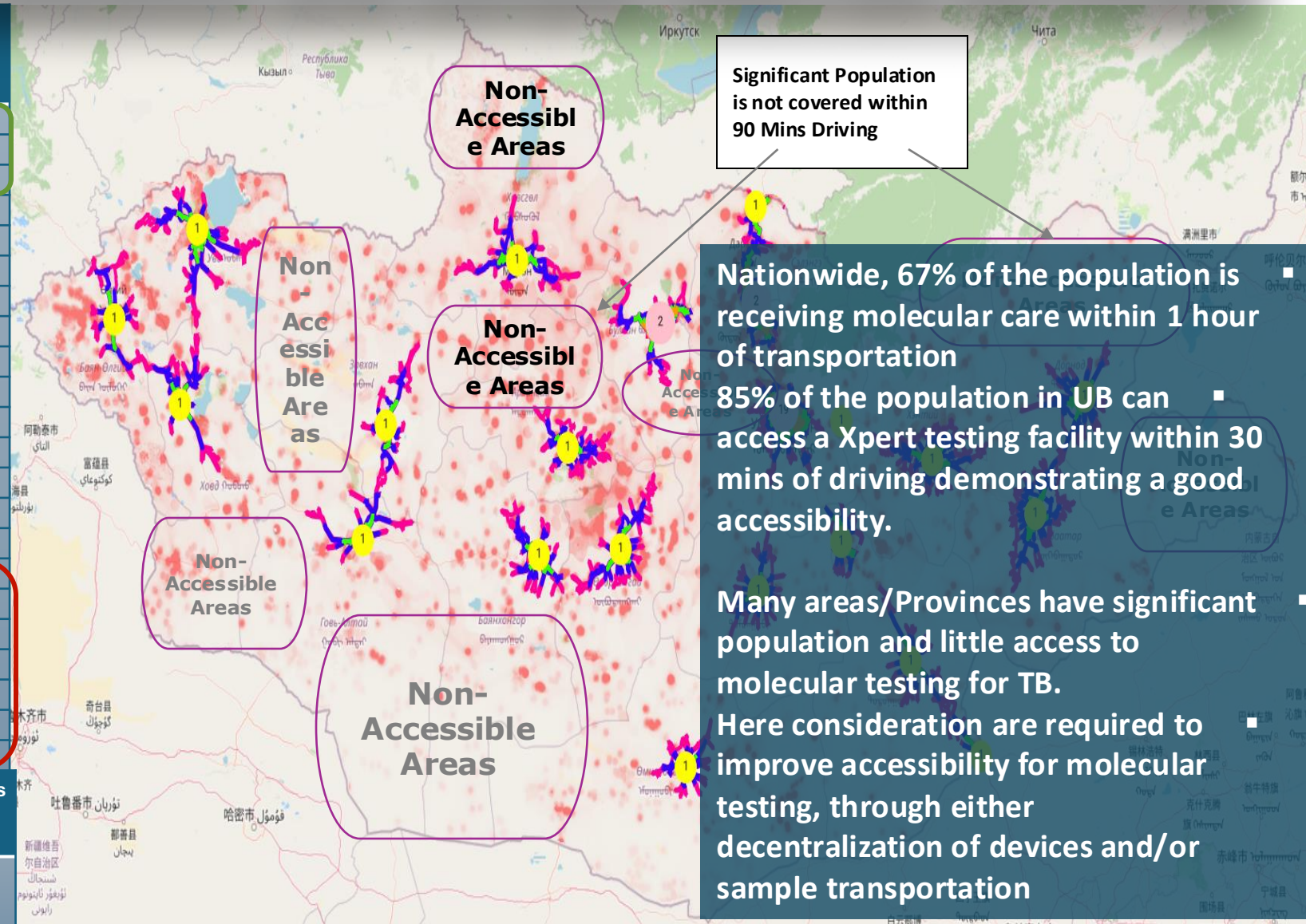
- ❑ High TB burden country: Incidence rate = **491 per 100,000** (increasing trend)
- ❑ High MDR-TB burden country: Proportion = **6.67% of new cases**, (increasing trend)
- ❑ Low treatment coverage rate – **18%**
- ❑ Low treatment success rate (TB) – **87%, 2022**



Scope for improving access to molecular TB testing through decentralization and sample referrals services

Name	Total Population	% 30-mins Driving Time	% 60-mins Driving Time	% 90-mins Driving Time
Ulaanbaatar	1882903	85%	94%	98%
Orkhon	114834	66%	79%	87%
Darkhan-Uul	106571	53%	72%	77%
Govisumber	15064	33%	57%	71%
Dornod	66485	32%	47%	50%
Bayankhongor	72625	31%	40%	44%
Bayan-Ölgii	85463	30%	36%	42%
Sükhbaatar	49755	34%	36%	39%
Khovd	71877	22%	30%	37%
Dornogovi	68526	21%	29%	39%
Hovsgel	117755	21%	29%	35%
Selenge	97081	18%	28%	42%
Övörkhangai	97252	20%	25%	30%
Bulgan	48147	19%	23%	26%
Ömnögovi	84418	21%	23%	27%
Zavkhan	50365	18%	23%	28%
Töv	76907	17%	20%	25%
Uvs	63346	12%	19%	27%
Arkhangai	78222	13%	18%	26%
Dundgovi	30938	10%	17%	25%
Govi-Altai	47739	11%	14%	18%
Khentii	64298	2%	11%	23%

Name	Total Population	% 15-mins Driving Time	% 30-mins Driving Time	% 60-mins Driving Time	% 90-mins Driving Time
Mongolia	3.4M	46%	59%	67%	73%



Overview of the National Strategic Plan for Tuberculosis (2025–2028) and TB health system optimization

Components of TB health system optimization

- ❑ **Decentralize TB diagnostic and treatment** by enabling rapid testing at PHC units (family and soum health centers).
- ❑ **Establish mandatory TB case notification across all health care levels.**
- ❑ **Initiate treatment immediately** after case detection.
- ❑ **Provide continues training** for doctors and health workers at **all health care levels.**
- ❑ **Intensify active case finding in high-risk groups** through early detection with X-ray and rapid molecular testing.
- ❑ **Screen for TB at all health care levels**, regardless of patient visit reason.
Routine TB screening of >90% visitors of >90% health facilities
- ❑ **Integrate TB detection into preventive check-ups and early screening programs.**
- ❑ **Expand multisectoral collaboration** for community-led TB service monitoring and strengthen oversight.

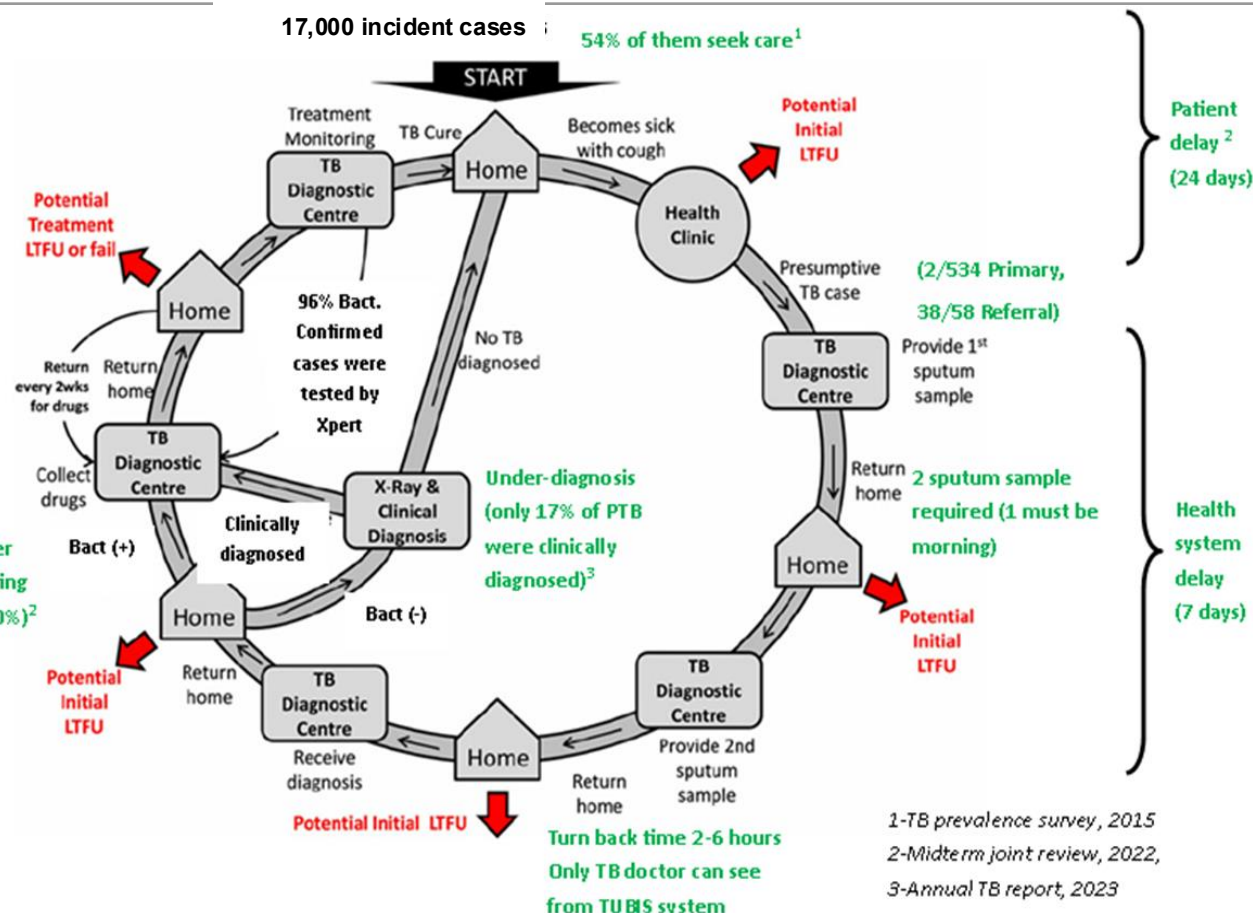
Health system optimization for TB care & services in Mongolia

Dr Socorro Escalante

WHO Representative to Mongolia



Gaps in the TB Cascade of Care

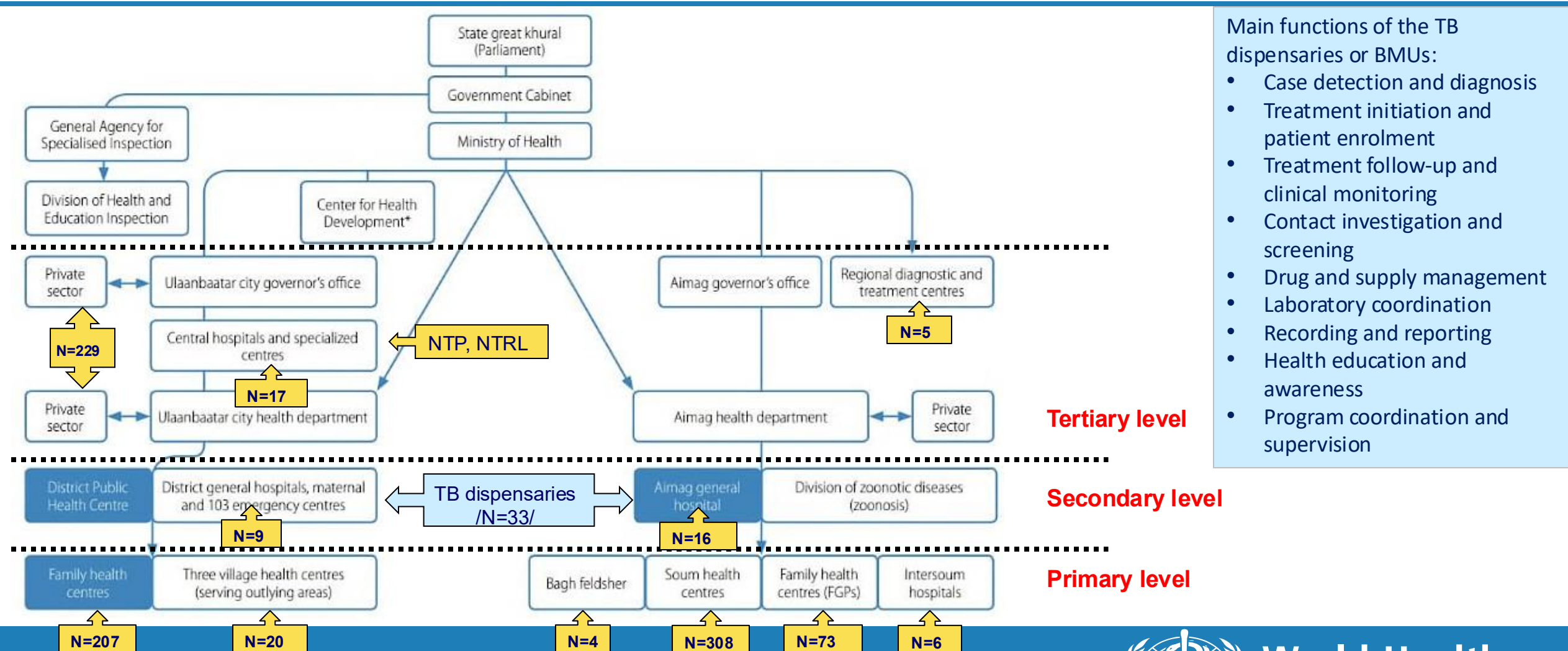


Key points:

- ❑ **Only 54% of patients seek care**
- ❑ **Patient delays** (time from the onset of TB-related symptoms until the first contact with a healthcare provider) – **24 days**
- ❑ **Health system delays** (time from that first healthcare visit to the initiation of anti-TB treatment) – **7 days**
- ❑ **Underreporting of cases – 15-30%**
- ❑ **Catastrophic costs = 69% of TB affected families**
 - ❑ 85% (median =\$1,328) of DR-TB-affected households
 - ❑ 65% (median =\$500) of DR-TB-affected households

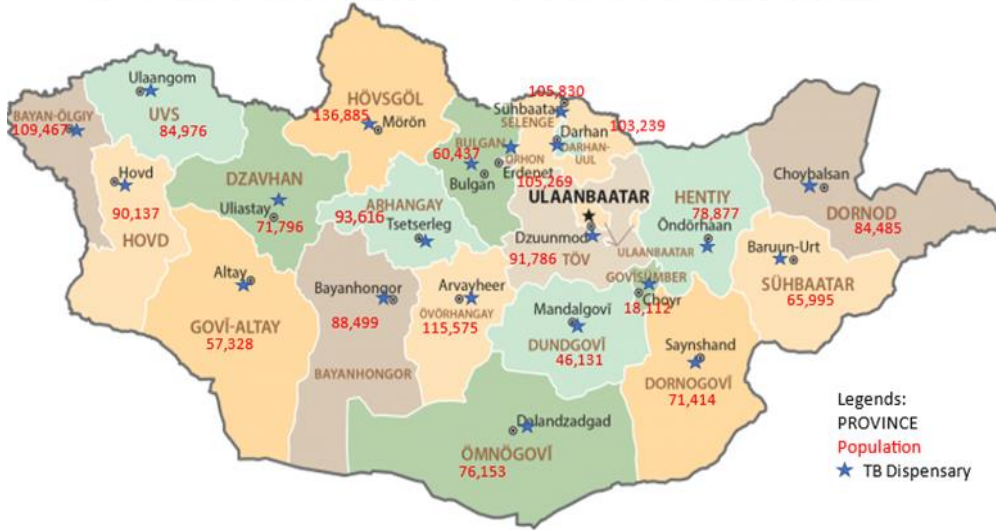
TB catastrophic cost survey in Mongolia, 2017

Structure of Health care system



Centralized TB Dispensary System leading to significant drop-offs at every stage of the TB cascade

TB Services in Mongolia– 21 Province TB Dispensary



Key points:

- ❑ TB diagnosis & treatment is centralized in one dispensary per province/district
- ❑ These specialized, facility-based services are **not easily accessible**.
- ❑ **Primary health care facilities are underutilized** for TB screening/diagnosis, resulting in missed cases or delayed diagnosis and treatment initiation.
- ❑ **Patients face long travel times and multiple visits**, which causes delays & treatment drop-outs.

This centralized service model leads to significant drop-offs at every stage of the TB cascade — from symptom recognition, diagnosis, and treatment initiation, to treatment completion.

Result: An estimated 80% of TB cases remain undetected and untreated, perpetuating ongoing transmission in the community.

Health system optimization for TB care and services

This approach involves a major policy and operational shift towards **optimizing the health system for** TB services, horizontally and vertically , making care more accessible, increasing TB case notifications, improving patient outcomes and reducing the burden on specialized TB facilities..

This approach focuses on:

- **Expanding TB screening to outpatient departments** in state national referral hospitals, private health facilities, and PHC facilities (Family Health Centres, and Soum Health Centres) with the aim to find more TB cases.
- **Enhancing diagnostic capacities at different health system levels**, particularly in PHC settings, to increase TB case notifications.
- **Introducing Digital Adherence Technologies** to improve patients' access and adherence to TB treatment, ultimately improving the treatment success rate.
- Introducing other **digital tools for case-based surveillance and contact tracing**
- **Expanding community-based care and follow-up** to improve treatment adherence and reduce drop-outs.

Current system				
Level	Case finding	Diagnosis	Treatment	TPT
Tertiary	-/+	-/+	-	-
Secondary	-/+	-	-	-
Secondary TB	+++	+++	+++	+++
PHC	+	+	+/-	-
Private	+	-	-	-



Health system optimization				
Level	Case finding	Diagnosis	Treatment	TPT
Tertiary	+++	+++	+	-
Secondary	+++	+++	-	-
Secondary TB	++	++	+	-
PHC	+++	++	+++	+++
Private	+++	++	+	+

By shifting from a centralized, specialized approach to a community-based and people-centered model at the PHC level, Mongolia aims to increase TB case identification, elevate treatment coverage rate to over 70%, and boost treatment success rate to 90%.

Thank you for attention